

As a developmental neuropsychologist and cross-cultural psychologist, Dr. Prescott has been concerned with the consequences of pain, and pleasure deprivations, upon the developing brain and social-sexual behaviors. He explained:

There is now formidable systematic scientific data that documents the reciprocal nature of pain and pleasure (pleasure inhibits pain and violence); and that pleasure deprivation leads to the breakdown of this normative reciprocal principle of brain function. One consequence of this breakdown is the development of sado-masochistic behaviors, e.g., self-mutilations, self-flagellation, and propensity to inflict pain upon others, specifically, sexual violence, torture and mutilation.

He went on to ask:

What are the neuropsychological consequences of inflicting extraordinary painful stimuli on those formative brain systems that are designed to experience pleasure, re the genitals? How does the brain react to and process genital pleasure when that pleasure is buried in a neuronal background of genital pain that is stored in the 'unconscious brain' from repressed genital and sexual mutilations? Does this infusion and confusion of genital pain and pleasure contribute to the formation of the sado-masochistic personality or, at least, impair the ability to realize the higher cognitive and spiritual dimensions of sexual pleasure and love?

Research by other investigators indicates we need to consider Prescott's concerns with grave seriousness.

Dr. Lee Salk and his associates found a significant relationship between adolescent suicides and prenatal and perinatal stress (Lancet 1985; 624-627). Dr. Bertil Jacobsen's research revealed an undeniable association between birth trauma and adult self-destructive behavior ("Perinatal Origin of Adult Self-Destructive Behaviors," Acta psychiatr. scand. 1987; 76:364-371).

Jacobsen agrees that circumcision trauma could possibly produce self-destructive behavior. This, he says, could be tested by comparing the circumcision rates among violent suicide victims (by firearms, jumping from heights, hanging, etc.) and non-violent suicide victims (poisoning and drowning). We're looking for researchers to help us with such a study. Hard evidence would certainly be valuable in evaluating the consequences of circumcission.

In the meantime, the anecdotal reports with regard to circumcision trauma tell their own story. There are too many now to be ignored. I have heard from more than 5,000 men who consider themselves to be victims of circumcision and until recently have suffered in silence and humiliation.

Several of these men claim never to have forgotten their circumcision experience. "It was like a giant bird landed on my stomach and disemboweled me with his talons. I can still feel the excruciating pain." Others have relived their circumcisions through hypnotherapy and psychotherapy. All say they don't remember in words, but with feelings.

Those who tell me that we can't remember circumcision (and therefore it has no psychological effect) don't realize that on an unconscious level they remember circumcision very well. Ten years ago, when I was suffering from sexual impotence and depression, I entered Primal Therapy, a process where childhood and infantile memories are dragged up and relived. In the second year of my therapy I relived an